PATHOLOGY · MCQ SET

Stomach I — Gastritis & Peptic Ulcer Disease

SGL 1 · part 1 of 2 · Dr Jawher Tahir Omer
GIT BLOCK · PRACTICE MCQs

Pathology GIT Block · MEDucation by Mattin Majid · Practice MCQs

HOW TO USE THIS SET

20 questions, mixed difficulty, 9 case-based. Attempt every question before checking the answer key on the last page.

4 Easy | 10 Medium | 6 Hard

1. EASY

An acute gastric ulcer occurring after severe burns is called:

A. Cushing ulcer
B. Curling ulcer
C. Stress ulcer
D. Meckel ulcer
E. Boerhaave ulcer

2. EASY

Which organism is most commonly responsible for chronic gastritis overall?

A. Helicobacter pylori
B. Candida albicans
C. Trypanosoma cruzi
D. Herpes simplex virus
E. Epstein-Barr virus

3. EASY

Duodenal ulcers are how many times more common than gastric ulcers?

A. Equally common
B. Twice as common
C. Four times as common
D. Ten times as common
E. Gastric ulcers are more common

4. EASY

What is the commonest complication of peptic ulcer disease?

A. Perforation
B. Malignant transformation
C. Haemorrhage
D. Pyloric obstruction
E. Fistula formation

5. MEDIUM CASE

A gastric biopsy shows loss of surface epithelium that does not extend below the muscularis mucosa.

This finding is best classified as:

A. Ulceration
B. Erosion
C. Intestinal metaplasia
D. Atrophic gastritis
E. Dysplasia

6. MEDIUM CASE

A patient in septic shock on the ICU develops multiple small, superficial gastric mucosal defects with oedema and congestion, none extending below the muscularis mucosa.

Which named eponym, if any, applies to lesions of this type in a shocked patient?

A. Curling ulcer
B. Cushing ulcer
C. Stress ulcer
D. No eponym applies — this is simple erosive gastritis, and eponyms apply only to full ulcers
E. Meckel ulcer

7. MEDIUM CASE

A 62-year-old woman has megaloblastic anaemia. Gastric biopsy shows mucosal atrophy with no organisms seen on silver stain, and serum shows achlorhydria.

Which mechanism best explains her anaemia?

A. H. pylori-induced hypochlorhydria impairing B12 absorption
B. Antibodies against parietal cells causing loss of intrinsic factor
C. Chronic NSAID-induced mucosal bleeding
D. Chemical reactive gastritis from bile reflux
E. Iron deficiency secondary to occult blood loss

8. MEDIUM

Why does H. pylori gastritis not typically produce pernicious anaemia, while autoimmune gastritis does?

A. H. pylori infects the duodenum, not the stomach
B. H. pylori does not cause achlorhydria, since parietal cell damage is far less severe than in autoimmune gastritis
C. H. pylori destroys chief cells rather than parietal cells
D. Autoimmune gastritis does not affect intrinsic factor production
E. Pernicious anaemia requires a bacterial trigger, which autoimmune gastritis lacks

9. MEDIUM CASE

A gastric biopsy from a patient without H. pylori on silver stain shows extensive gastric atrophy with endoscopically visible submucosal vessels.

Given that H. pylori is absent, what is the most likely underlying process?

A. Chemical (reactive) gastritis
B. Autoimmune gastritis — the commonest cause of gastric atrophy in H. pylori-negative patients
C. NSAID-induced acute gastritis
D. Eosinophilic gastritis
E. Normal ageing change requiring no further work-up

10. MEDIUM

H. pylori infection confined to the antrum tends to raise acid production. What is the most direct downstream consequence of this pattern?

A. Achlorhydria
B. Pernicious anaemia
C. Increased risk of duodenal ulcer
D. Loss of intrinsic factor
E. Gastric atrophy with intestinal metaplasia

11. MEDIUM CASE

A different pattern of H. pylori infection, involving pangastritis progressing to multifocal atrophic gastritis, is identified in another patient.

This pattern is on the pathway toward which endpoint?

A. Duodenal ulcer
B. Gastric adenocarcinoma, via intestinal metaplasia
C. Pernicious anaemia
D. Curling ulcer
E. Boerhaave syndrome

12. MEDIUM

Which histological finding is characteristic of H. pylori gastritis on silver stain and routine histology?

A. Spiral bacilli with intraepithelial and lamina propria neutrophils, plus lymphoid aggregates
B. Endoscopic whitish discoloration with visible vessels and no organisms
C. Non-necrotizing granulomas
D. Signet ring cells infiltrating the lamina propria
E. Eosinophils numerous and far from the antrum

13. MEDIUM CASE

A duodenal ulcer biopsy is taken from the anterior wall a few centimetres from the pylorus.

Compared with a gastric ulcer of similar size, this duodenal ulcer is:

A. More likely to undergo malignant transformation
B. Less likely to undergo malignant transformation
C. Equally likely to undergo malignant transformation
D. Guaranteed to be H. pylori-negative
E. Located preferentially on the posterior wall instead

14. MEDIUM

Histologically, an established peptic ulcer shows four layers from the lumen down. What is the correct order?

A. Granulation tissue, necrotic debris, inflammatory infiltrate, scar
B. Necrotic debris, inflammatory infiltrate, granulation tissue, scar
C. Inflammatory infiltrate, necrotic debris, scar, granulation tissue
D. Scar, granulation tissue, inflammatory infiltrate, necrotic debris
E. Necrotic debris, scar, inflammatory infiltrate, granulation tissue

15. HARD CASE

A trauma patient in the ICU with severe head injury develops acute gastric ulceration. A separate ICU patient with 60% body-surface burns develops a similar picture.

Which eponym pair correctly matches each patient?

A. Head injury → Curling; burns → Cushing
B. Head injury → Cushing; burns → Curling
C. Both are correctly called stress ulcers only, since eponyms apply exclusively to NSAID-induced lesions
D. Head injury → stress ulcer; burns → Cushing ulcer
E. Neither qualifies for an eponym since neither is in shock

16. HARD

Which statement about autoimmune gastritis is TRUE?

A. It accounts for the majority of chronic gastritis cases overall
B. It is the commonest cause of gastric atrophy despite being under 10% of chronic gastritis cases
C. It characteristically shows spiral bacilli on silver stain
D. It causes hypochlorhydria rather than achlorhydria
E. It has no association with malignancy risk

17. HARD CASE

A patient with longstanding H. pylori gastritis has progressed through pangastritis and multifocal atrophic gastritis to intestinal metaplasia on recent biopsy.

Compared with a patient whose H. pylori infection remains confined to the antrum with preserved acid output, this patient's dominant future risk has shifted toward:

A. Duodenal ulcer, since both patterns raise DU risk equally
B. Gastric adenocarcinoma, as the atrophic/metaplastic route replaces the acid-driven DU route
C. Curling ulcer
D. Pernicious anaemia, since intestinal metaplasia always indicates autoimmune gastritis
E. Boerhaave syndrome

18. HARD

Malignant transformation of a peptic ulcer is a rare but recognised complication. It is more likely to occur in:

A. Duodenal ulcers, because they are far more numerous overall
B. Gastric ulcers, even though DU are more common overall
C. Ulcers caused by NSAIDs specifically, regardless of site
D. Stress ulcers only
E. Curling ulcers only

19. HARD CASE

A biopsy from a chronic gastric ulcer shows tissue loss extending below the muscularis mucosa, with a base showing, from the lumen inward: necrotic debris, then an inflammatory infiltrate, then granulation tissue, then a fibrous scar.

Which single feature in this description confirms this is an ulcer rather than an erosion, independent of the layered histology?

A. The presence of necrotic debris at the surface
B. The presence of an inflammatory infiltrate
C. Tissue loss extending below the muscularis mucosa
D. The presence of granulation tissue
E. The presence of a fibrous scar

20. HARD CASE

A patient has H. pylori confirmed on biopsy. One year later, repeat biopsy after eradication therapy still shows gastric atrophy and intestinal metaplasia, though the organism is no longer present.

What is the most accurate statement about this patient's cancer risk going forward?

A. Eradicating H. pylori has fully reversed his cancer risk back to baseline
B. The atrophy and metaplasia already present may persist and continue to carry increased cancer risk even after successful eradication
C. Eradication therapy itself is a novel risk factor for gastric cancer
D. His risk is now lower than someone who was never infected
E. Metaplasia always reverts to normal mucosa within a year of eradication

ANSWER KEY & RATIONALE

Don't peek until you've committed to an answer on every question.

Q	Ans	Why
1	**B**	Curling ulcer follows severe burns.
2	**A**	H. pylori is the most common cause of chronic gastritis overall.
3	**C**	Duodenal ulcers are four times more common than gastric ulcers.
4	**C**	Haemorrhage is the commonest complication of PUD.
5	**B**	Loss confined above the muscularis mucosa is, by definition, erosion, not ulceration.
6	**C**	Multiple superficial gastric lesions in a shocked/septic patient are stress ulcers — the generic eponym for shock, sepsis, or trauma.
7	**B**	Autoimmune gastritis destroys parietal cells, removing intrinsic factor and causing achlorhydria — the mechanism behind pernicious (megaloblastic) anaemia.
8	**B**	H. pylori can cause hypochlorhydria but not the severe parietal cell destruction (achlorhydria) that autoimmune gastritis produces — hence no pernicious anaemia.
9	**B**	Autoimmune gastritis is under 10% of chronic gastritis overall but is the commonest cause of gastric atrophy, and the commonest form in H. pylori-negative patients.
10	**C**	Antral H. pylori gastritis raises acid production, increasing duodenal ulcer risk — the acid-driven route, distinct from the atrophic/metaplastic route.
11	**B**	Pangastritis → multifocal atrophic gastritis → intestinal metaplasia is the pathway toward gastric adenocarcinoma.
12	**A**	Spiral bacilli on Warthin-Starry silver stain with neutrophilic infiltrate and lymphoid aggregates are the histological hallmark of H. pylori gastritis.
13	**B**	Malignant change is less likely in duodenal ulcers than gastric ulcers, even though both remain rare overall.
14	**B**	The four layers from the lumen down are: necrotic debris, inflammatory infiltrate, granulation tissue, then scar (fibrosis).
15	**B**	Cushing ulcer follows intracranial disease (head injury); Curling ulcer follows severe burns.
16	**B**	Autoimmune gastritis is under 10% of chronic gastritis cases but is the leading cause of gastric atrophy overall.
17	**B**	The atrophic/intestinal metaplasia route (from pangastritis) drives gastric adenocarcinoma risk, distinct from the acid-driven antral route that raises DU risk.
18	**B**	Malignant transformation of a benign ulcer, though rare in both, is more likely in gastric ulcers than duodenal ulcers.
19	**C**	Depth below the muscularis mucosa is the defining feature separating ulceration from erosion — the layered histology describes an established ulcer, not the defining criterion itself.
20	**B**	Established atrophy and intestinal metaplasia can persist and continue to carry elevated cancer risk even after H. pylori is successfully eradicated, which is why ongoing surveillance is still warranted.

Recalled and source questions are treated as evidence of what gets asked, not as a source of correct answers — verify against your lecture notes.